



LIFECARE LIVING SOLUTIONS

Care Management System

Feature Overview
for Care Facility Teams

PLATFORM

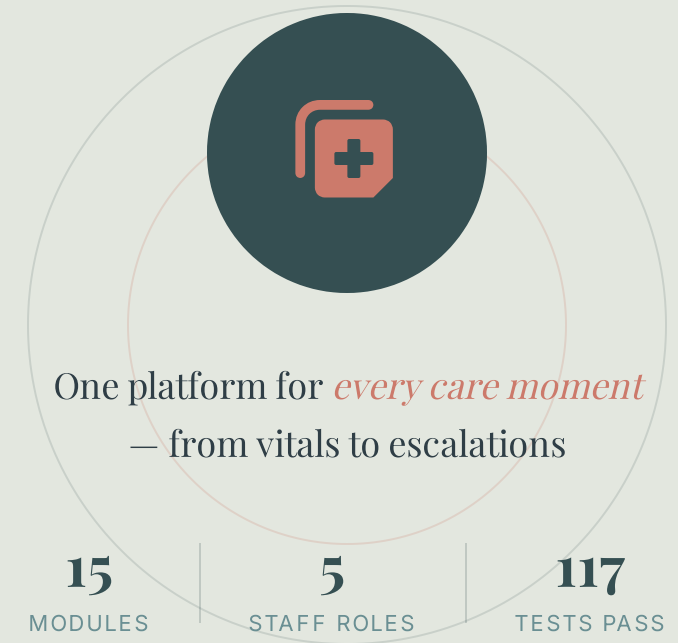
LifeCare CMS

AUDIENCE

All Staff Roles

MODULES

15 Core Modules



LifeCare CMS gives every role the right tools

PLATFORM OVERVIEW

WHAT THE SYSTEM COVERS

A purpose-built care management platform for residential care facilities — replacing paper-based workflows with a secure, role-aware digital system accessible from any device.

- **Role-based access** — 5 roles with distinct permissions and views
- **PIN-based authentication** — no passwords to forget
- **Works on any device** — desktop, tablet, and mobile browsers
- **Secure by design** — resident data is never publicly accessible
- **15 integrated modules** — from vitals to escalations to inventory

STAFF ROLES



Administrator

Full system access, approvals, audit



Care Manager

Clinical oversight, approvals, incidents



Nurse

Vitals, MAR, SBAR, endorsements, tasks



Caregiver

Care logs, task execution, alerts



Receptionist

Resident registration, visitor management, appointments

Resident Management — every resident's story in one place

KEY CAPABILITIES

Each resident has a complete digital profile that serves as the starting point for all care documentation and clinical decisions.

- ✓ **Full profile:** personal info, diagnosis, allergies, care level, room
- ✓ **Emergency contacts** always one tap away
- ✓ **Status tracking:** Active, Discharged, On Leave, Deceased
- ✓ **Instant search** by name or room number
- ✓ **Edit rights** restricted to Care Manager and Administrator



Maria Santos

Room 204 · Admitted: Jan 12, 2024 · Age 78

ACTIVE

PRIMARY DIAGNOSIS Hypertension, T2DM	CARE LEVEL Assisted Care	ALLERGIES Penicillin, Sulfa
PRIMARY PHYSICIAN Dr. Reyes	EMERGENCY CONTACT Juan Santos (Son)	DIET RESTRICTION Low sodium, diabetic

 Search by name or room number

 Full care history timeline

 Role-restricted edit access

Vital Signs Monitoring — abnormal values trigger alerts automatically

MODULE 02



BLOOD PRESSURE

90–140 / 60–90 mmHg

NORMAL



HEART RATE

60–100 bpm

NORMAL



SPO₂

≥ 95%

ALERT < 90%



TEMPERATURE

36.1–37.2 °C

NORMAL



RESP. RATE

12–20 /min

NORMAL



WEIGHT

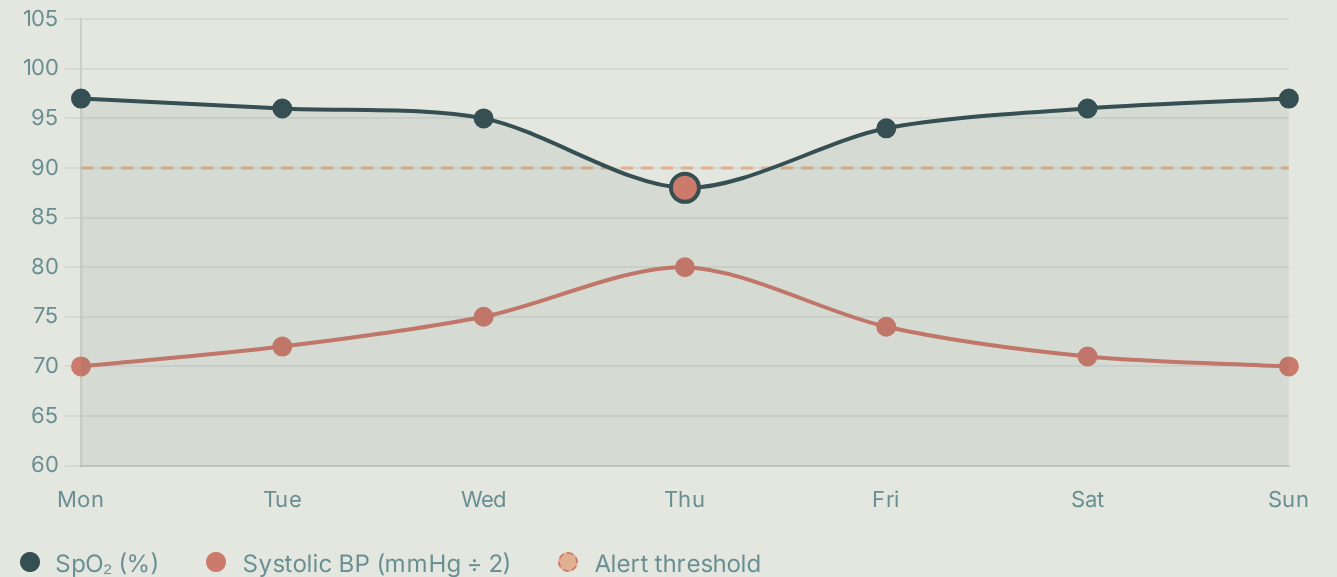
Logged per shift

TRACKED

KEY CAPABILITIES

- ✓ **6 vital parameters** logged per resident per shift
- ✓ **Automatic alert generation** on abnormal values — no manual flagging
- ✓ **Historical trend chart** per resident — visualise changes over time
- ✓ **SLA countdown timer** on every alert — ensures timely response
- ✓ **Shift completeness tracking** — see which residents still need vitals

SAMPLE: SPO₂ & BLOOD PRESSURE — 7-DAY TREND



Medication Administration Record — every dose accounted for

KEY CAPABILITIES

The digital replacement for the paper medication chart — nurses document every scheduled dose with a timestamp, their name, and the outcome.

- ✓ **Dose sign-off:** Given / Refused / Held with reason
- ✓ **Missed dose detection** — auto-alert if window passes
- ✓ **Medication profile:** drug, dose, route, schedule, prescriber
- ✓ **Compliance report** per resident
- ✓ **Approval workflow** — new prescriptions require Care Manager sign-off

Maria Santos — Room 204

Morning Shift · May 22, 2026

MEDICATION	DOSE / ROUTE	SCHEDULE	STATUS	ADMINISTERED BY
Amlodipine Antihypertensive	5mg / PO	8:00 AM	GIVEN	N. Reyes, RN
Metformin Antidiabetic	500mg / PO	8:00 AM	GIVEN	N. Reyes, RN
Furosemide Diuretic	40mg / PO	10:00 AM	REFUSED	N. Reyes, RN
Atorvastatin Statin	20mg / PO	8:00 PM	HELD	—

SHIFT COMPLIANCE

87%

Approval workflow: New prescriptions require Care Manager sign-off before activation in the MAR

Care Logging — structured documentation across 7 domains

PER SHIFT · PER RESIDENT

Every shift, nurses and caregivers document care delivered across seven clinical domains. Each domain uses structured, clinically relevant fields — eliminating free-text ambiguity and ensuring consistent, complete records.

7
CARE DOMAINS

3
SHIFTS / DAY

100%
COMPLETION TRACKED



Meals & Nutrition

Intake %, diet type, appetite, feeding assistance



Bowel & Bladder

Bristol Scale, frequency, continence, catheter



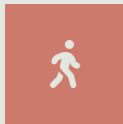
Mood & Behaviour

Mood state, behaviour, social engagement



Pain Assessment

0–10 scale, body location, pain type, action taken



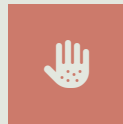
Mobility

Assistance level, repositioning, mobility aid used



Sleep Quality

Hours slept, quality rating, sleep disturbances



Skin Integrity

Condition, redness, wound observation, dressing



Shift completion score — nurses see documentation progress at a glance; incomplete domains are flagged



Care history timeline — scroll back through any previous shift for any resident



Accessible to Nurses and Caregivers — reviewed and monitored by Care Manager

Incident Reporting — from fall to skin breakdown, nothing undocumented

6 INCIDENT TYPES

	Fall Witnessed or unwitnessed, with injury assessment
	Skin Integrity Pressure ulcers, wounds, skin breakdown
	Behavioural Aggression, agitation, wandering events
	Medication Error Wrong dose, wrong drug, missed administration
	Environmental Hazards, equipment failure, safety concerns
	Other Any unplanned event not covered above

SEVERITY GRADING — AUTO-ALERT THRESHOLD

MINOR No alert	MODERATE Warning alert	SEVERE Auto-alert	CRITICAL Immediate alert
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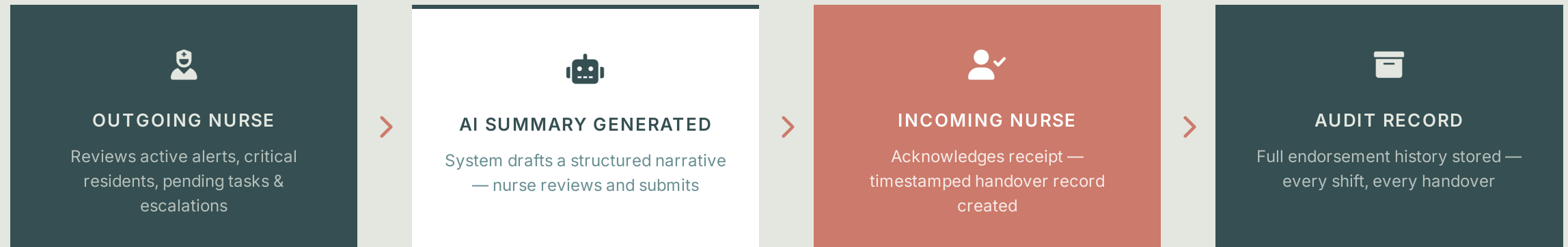
Severe and Critical incidents automatically notify the Care Manager and appear in the alert queue with an SLA countdown.

 Photo upload for wound or scene documentation	 Witness details and immediate action recorded
 Review and close workflow for Care Manager	 Printable PDF for regulatory submission

Shift Endorsement — structured handover, nothing lost between shifts

MODULE 08

HANDOVER PROCESS



- ✓ **Structured handover** covering alerts, critical residents, pending tasks, and open escalations
- ✓ **AI-generated narrative** ⚡ AI reduces documentation time significantly
- ✓ **Outgoing nurse reviews** and submits; incoming nurse formally acknowledges

- ✓ **Full endorsement history** per shift — complete, auditable handover chain
- ✓ **Eliminates verbal handover errors** — no missed alerts, no forgotten follow-ups
- ✓ **Timestamped records** for accountability and regulatory compliance

Alert Management — nothing critical goes unnoticed

AUTOMATIC ALERT SOURCES

**Vital Signs**
Abnormal BP, SpO₂, HR,
Temp, RR

**MAR**
Missed dose windows,
refused meds

**Care Logs**
Missed shift
documentation

**Incidents**
Severe and critical
incident reports

 **CRITICAL**
Immediate action

 **WARNING**
Timely response

 **INFO**
For awareness

ROLE-BASED ACCESS CONTROL

ROLE	ACKNOWLEDGE	SNOOZE	RESOLVE
Administrator	✓	✓	✓
Care Manager	✓	✓	✓
Nurse	✓	✓	✓
Caregiver	✓	✗	✗
Receptionist	✓	✗	✗

 **SLA countdown timer** on every alert — escalates automatically if not acknowledged within the response window. Enforced at both server and UI level.

SBAR Escalation Tool — structured clinical escalation in four steps

MODULE 10

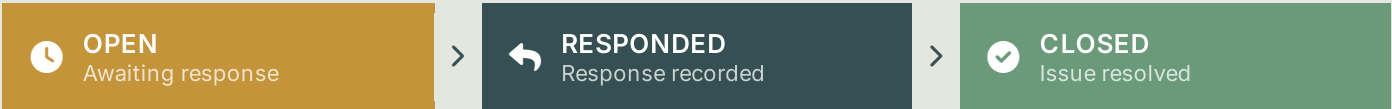
KEY CAPABILITIES

When a resident needs physician input or senior decision-making, nurses open a formal SBAR escalation — guiding them through a structured, clinically sound communication.

- ✓ **Background auto-filled** from resident's profile — conditions, allergies, care level
- ✓ **AI draft button**  generates recommendation from S, B, A fields
- ✓ **Directed to:** Physician, Family, Senior Nurse, or Care Manager
- ✓ **Response timer** — elapsed time shown on every open escalation
- ✓ **Linked to Physician Comms** — escalation and resulting call in one record
- ✓ **Printable SBAR form** for clinical records and handover

SBAR FORM — SAMPLE ESCALATION

S	SITUATION Mrs. Santos, Room 204, SpO ₂ dropped to 88% at 14:30. Respiratory rate 24/min, appears laboured.
B	BACKGROUND · AUTO-FILLED FROM RESIDENT PROFILE 78F, Hypertension, T2DM, Allergic to Penicillin. On Amlodipine 5mg OD. Assisted care level.
A	ASSESSMENT Possible respiratory compromise. Vitals trending down since morning shift. No fever, no chest pain reported.
R	RECOMMENDATION · AI DRAFT AVAILABLE Request urgent physician review. Consider O ₂ supplementation and chest X-ray. Increase monitoring frequency to 1-hourly.



Physician Communication Log — every call and visit on record

MODULE 11

SAMPLE LOG ENTRIES

PHONE CALL

Dr. Ramon Reyes

May 22, 2026 · 09:15 AM

Resident: Maria Santos, Room 204 · Logged by: N. Cruz, RN

REASON FOR CONTACT

SpO₂ dropped to 88% during morning vitals — requesting assessment and order review

INSTRUCTIONS RECEIVED

Administer O₂ at 2L/min via nasal cannula. Repeat SpO₂ in 30 minutes. If no improvement, transfer to ER.

FOLLOW-UP REQUIRED

Deadline: May 22, 2026 10:00 AM — Repeat SpO₂ check

IN-PERSON VISIT

Dr. Ana Lim

May 21, 2026 · 02:30 PM

Resident: Jose Ramos, Room 108 · Logged by: N. Santos, RN

OVERDUE

Follow-up was due May 21, 2026 6:00 PM — wound dressing change

SBAR Link:

Escalations can be directly linked to the resulting physician communication — one click to view the full clinical context

KEY CAPABILITIES

- ✓ Log every physician contact: phone, visit, written, telemedicine
- ✓ Full instructions received — verbatim, not summarised
- ✓ Follow-up tracking with deadline and completion confirmation
- ✓ Overdue follow-up badge — red indicator when deadline passes
- ✓ Encounter history per resident — full referral and communication trail
- ✓ Printable PDF for clinical records and regulatory submission

CONTACT METHODS

PHONE

IN-PERSON

WRITTEN

TELEMEDICINE

Task Assignment — nurses assign, caregivers execute, nothing falls through

MODULE
12

KEY CAPABILITIES

Nurses translate care plans into specific tasks assigned to caregivers with a category, priority, and due time. Caregivers confirm completion; nurses review the shift summary.

- ✓ **6 categories:** Personal Care, Hygiene, Medication, Mobility, Nutrition, Observation
- ✓ **Priority levels:** Routine, Urgent, Critical
- ✓ **Overdue badge** — red indicator with elapsed time for past-due tasks
- ✓ **My Tasks view** for caregivers — only their assigned tasks
- ✓ **Shift summary:** completion rate per caregiver for nurse review

BOARD VIEW — CURRENT SHIFT

PENDING2	IN PROGRESS1	COMPLETED3
Santos, M. · Rm 204 URGENT Reposition every 2 hours MOBILITYA. Cruz 🕒 38 min overdue OVERDUE	Ramos, E. · Rm 312 CRITICAL Oral medication administration MEDICATIONN. Reyes, RN Due 9:00 AM	Lim, C. · Rm 215 ROUTINE Breakfast assistance & intake log NUTRITIONA. Cruz ✅ Done 8:42 AM
Dela Cruz, R. · Rm 108 ROUTINE Assist with morning bath HYGIENEB. Santos Due 10:00 AM		Garcia, T. · Rm 101 ROUTINE Vital signs observation OBSERVATIONB. Santos ✅ Done 8:15 AM

 Shift summary shows completion % per caregiver

 My Tasks view for caregivers — filtered to their assignments only

 Overdue tasks flagged with elapsed time in real time

APPROVAL PROCESS

New prescriptions and appointment referrals require formal sign-off before becoming active — ensuring clinical decisions are reviewed by the right authority.



Nurse Submits Request

Medication prescription or appointment referral submitted with full clinical context



Care Manager Reviews

Full clinical context shown — resident, medication details, prescriber, or appointment type



Decision with Notes

Approve or reject with review notes and timestamp

✓ APPROVED

✗ REJECTED



Medication Prescriptions

- ✓ Saved as **inactive** until approved
- ✓ Activated in MAR **immediately** on approval
- ✓ Rejected items flagged to submitting nurse with reviewer notes
- ✓ Full approval history per resident for accountability



Appointment Referrals

- ✓ Referral saved as **pending** until approved
- ✓ Confirmed in appointment schedule on approval
- ✓ Specialist, clinic, date, and purpose all reviewed
- ✓ Outcome documented after the appointment



Care Manager



Administrator

Medication Inventory — real-time stock visibility with smart alerts

INVENTORY STOCK LIST — SAMPLE VIEW

🔍 Search by name or brand

🔴 Critical Stock

📅 Expiring in 30 days

✕ Clear (2)

MEDICATION	CURRENT QTY	STOCK LEVEL	EXPIRY DATE	LOCATION
Amlodipine 5mg Norvasc · Tablet	4 tabs	CRITICAL	Jun 15, 2026 Expiring in 24 days	Cabinet A-2
Metformin 500mg Glucophage · Tablet	0 tabs	OUT OF STOCK	Apr 30, 2026 Expired	Cabinet B-1
Losartan 50mg Cozaar · Tablet	18 tabs	LOW	Sep 10, 2026 4 months away	Cabinet A-1
Paracetamol 500mg Biogesic · Tablet	120 tabs	NORMAL	Dec 01, 2026 7 months away	Cabinet C-3

KEY CAPABILITIES

- ✓ **Stock badges:** Out of Stock, Critical, Low, Normal — color-coded at a glance
- ✓ **Expiry tracking** with Expired and Expiring Soon badges per item
- ✓ **Filter by stock level** — isolate critical items in one click
- ✓ **Filter by expiry** — 7-day, 30-day, or already expired
- ✓ **Adjust stock** — add or subtract quantity with reason note
- ✓ **Purchase Requests tab** — approve, reject, or mark ordered

⚡ AUTO PURCHASE REQUEST

When a stock adjustment causes quantity to fall **below the reorder threshold**, a purchase request is automatically created and queued for administrator review — no manual step required.

Medical Appointments — referrals tracked from request to outcome

MODULE 08



KEY CAPABILITIES

- ✓ **Specialist, clinic, date, and purpose** all captured at submission
- ✓ **Approval gate** — referrals require Care Manager sign-off before confirmation
- ✓ **Outcome documentation** — findings, follow-up instructions, and notes recorded after the visit
- ✓ **Full appointment history** per resident — chronological record of all referrals
- ✓ **Status tracking:** Pending, Confirmed, Completed, Cancelled



SAMPLE APPOINTMENT RECORDS

SPECIALIST

COMPLETED

Santos, Maria · Room 204

May 20, 2026

SPECIALIST

Dr. Ana Lim, Pulmonologist

CLINIC

St. Luke's Medical Center

PURPOSE

Respiratory evaluation — SpO₂ monitoring

APPROVED BY

N. Reyes, Care Manager

✓

Outcome: Mild COPD exacerbation. Prescribed Salbutamol inhaler PRN. Follow-up in 4 weeks. Chest X-ray ordered.

FOLLOW-UP

PENDING APPROVAL

Ramos, Eduardo · Room 108

May 25, 2026

SPECIALIST

Dr. Ramon Reyes, Cardiologist

SUBMITTED BY

N. Cruz, RN

🕒

Awaiting Care Manager review — will be confirmed in appointment schedule upon approval

LIFECARE CMS

One Platform. Every Care Moment.

13 modules. Every role. Every shift. Every resident.



NURSES

Document faster, escalate with confidence, never miss a critical event



CAREGIVERS

See only what's assigned, confirm completion, stay focused on residents



CARE MANAGERS

Approve, review, and oversee care decisions from a single queue



ADMINISTRATORS

Full visibility across all residents, staff, inventory, and compliance records



RECEPTIONISTS

Resident directory, visitor management, and appointment coordination

COMING SOON



Wound Care Tracker — stage progression, wound photos, dressing schedule



Audit Log — full chronological record of every staff action



Risk Scoring — Braden Scale, Morse Fall Scale, auto care plan triggers